

SUSPECT ADVERSE REACTION REPORT										

I. REACTION INFORMATION

1. PATIENT INITIALS (first, last) *****	1a. COUNTRY China - CN	2. DATE OF BIRTH			2a. AGE	3. SEX	3a. WEIGHT	4-6 REACTION ONSET			8-12 CHECK ALL APPROPRIATE TO ADVERSE REACTION
		Day **	Month ***	Year ****	69 Years	Male	68.5 kg	Day 30	Month Sep	Year 2023	
7 + 13 DESCRIBE REACTION(S) (including relevant tests/lab data) Event Verbatim [LOWER LEVEL TERM] (Related symptoms if any separated by commas) Bullous pemphigoid [Bullous pemphigoid (10006567)*], Seriousness: Life threatening, Outcome: recovering/resolving Information has been received from the investigator regarding a 69-year-old Asian (Chinese) male trial participant enrolled in a study entitled "An Open-label, Randomized Phase 3 Study to Evaluate Efficacy and Safety of Pembrolizumab (MK-3475) in Combination with Belzutifan (MK-6482) and Lenvatinib (MK-7902), or MK-1308A in Combination with Lenvatinib, versus (...) (Continued on Additional Information page)										☐ PATIENT DIED ☒ INVOLVED OR PROLONGED INPATIENT HOSPITALISATION ☐ INVOLVED PERSISTENCE OR SIGNIFICANT DISABILITY OR INCAPACITY ☒ LIFE THREATENING ☐ CONGENITAL ANOMALY ☐ OTHER	

II. SUSPECT PRODUCT(S) INFORMATION

14. SUSPECT PRODUCT(S) (include generic name) #1) BELZUTIFAN Tablet (belzutifan) - Batch/Lot #: ; #2) LENVATINIB MESYLATE Capsule (lenvatinib mesylate) - Batch/Lot #: ; ; ;		20. DID REACTION ABATE AFTER STOPPING DRUG? ☒ Yes ☐ No ☐ N/A
15. DAILY DOSE(S) #1) 120 mg 120 mg QD, every 1 days; 120 mg 120 mg QD, every 1 days (...)	16. ROUTE(S) OF ADMINISTRATION #1) Oral; Oral #2) Oral; Oral; Oral; Oral	
17. INDICATION(S) FOR USE #1) Advanced Clear Cell Renal Cell Carcinoma (ccRCC) [Clear cell renal cell carcinoma (10073251)*] (...)		21. DID REACTION REAPPEAR AFTER REINTRODUCTION? ☐ Yes ☐ No ☒ N/A
18. THERAPY DATE(S) (from/to) #1) 18-Oct-2023 to 07-Sep-2025; 07-Sep-2023 to 17-Oct-2023 (...)	19. THERAPY DURATION #1) 1.89 years; 41 days #2) 1.627 years; ongoing; (...)	

(Continued on Additional Information page)

III. CONCOMITANT PRODUCT(S) AND HISTORY

22. CONCOMITANT PRODUCT(S) AND DATES OF ADMINISTRATION (exclude those used to treat reaction) #1) Concomitant Product: Compound calcium carbonate (Calcium carbonate; Colecalciferol) - Therapy Date(s) (from/to): Jul-2023 to -- - Dose(s): 600 mg 600 mg, QD, strength: 600 mg, every 1 days - Indication(s) for use: prevention of osteoporosis [Osteoporosis prophylaxis (10049904)*]
23. OTHER RELEVANT HISTORY (e.g. diagnostics, allergies, pregnancy with last month of period, etc.) hemorrhoid [Hemorrhoids (10019611)*], 2018 to -- (continuing) Hb decreased [Haemoglobin decreased (10018884)*], 2023 to -- (continuing) Platelet count increased [Platelet count increased (10051608)*], 2023 to -- (continuing) (...)

(Continued on Additional Information page)

IV. MANUFACTURER INFORMATION

24a. NAME AND ADDRESS OF MANUFACTURER Merck Sharp & Dohme LLC noreply@merck.com		25b. NAME AND ADDRESS OF REPORTER ***** ***** Physician ***** *****, Beijing Beijing, ***** China	
	24b. MFR CONTROL NO. CN-009507513-2328416	26. REMARKS Worldwide UID - CN-009507513-2328416 Study Id - 6482-012 Patient ID - 419353 Center ID - 6011	
24c. DATE RECEIVED BY MANUFACTURER 11-Dec-2025	24d. REPORT SOURCE ☒ STUDY ☐ LITERATURE ☒ HEALTH PROFESSIONAL		
DATE OF THIS REPORT 23-Dec-2025	25a. REPORT TYPE ☐ INITIAL ☒ FOLLOWUP		

I. Reaction Information

7 + 13 DESCRIBE REACTIONS(S) (including relevant tests/lab data): Pembrolizumab and Lenvatinib, as First-line Treatment in Participants with Advanced Clear Cell Renal Cell Carcinoma (ccRCC)".

The trial participant's medical history was not reported. His concurrent conditions included Hemorrhoids, hemoglobin (Hb) decreased, and platelet count increased. Concomitant medications include calcium carbonate (+) cholecalciferol.

On 06-SEP-2023, the trial participant was randomized into the Arm A of the study protocol. On 07-SEP-2023, at 10:45, the trial participant started the study protocol with pembrolizumab (MK-3475) injection, at dose of 400 mg administered intravenously (IV) every 6 weeks (Q6W) up to 18 cycles; at 11:51, with belzutifan (MK-6482) tablet, 120 mg administered orally (PO) once daily (QD); and at 12:53, with lenvatinib mesylate (MK-7902) capsule, 20 mg administered PO QD. All study therapies were administered for the treatment of Advanced Clear Cell Renal Cell Carcinoma (ccRCC). That was also the only and last dose prior to the event onset of pembrolizumab (MK-3475) (Cycle 1), and on 30-SEP-2023, the last doses prior to the event onset of belzutifan (MK-6482) and lenvatinib mesylate (MK-7902) were administered to the trial participant (both at Cycle 1).

On 30-SEP-2023, the trial participant experienced bullous pemphigoid (Grade 4). On 11-OCT-2023, physical examination during C1D36 visit revealed scattered skin fissures on both hands and feet, accompanied by peeling. The trial participant reported experiencing skin cracks and peeling on both hands and feet (starting from 30-SEP-2023), accompanied by pain at the crack site. The trial participant had self-prepared vitamin E urea ointment for application treatment. Recorded as AE Hand Foot Syndrome, Grade 2, related to lenvatinib mesylate (MK-7902), unrelated to other investigational drugs, tolerable, continued to take lenvatinib mesylate (MK-7902) capsules without adjusting dosage. On 06-NOV-2023, hemoglobin was 103 g/L. On 27-NOV-2023, hemoglobin was 100 g/L. On 18-DEC-2023, hemoglobin was 100 g/L. On 20-DEC-2023, at the C3D22 visit, the trial participant reported worsening hand pain from the same day and was assessed to be related to hand foot syndrome. The AE of hand foot syndrome was updated to grade 2 and was intolerable. Lenvatinib mesylate (MK-7902) was temporarily suspended from 20-DEC-2023. On 04-JAN-2024, on the unplanned visit the trial participant reported that the pain in his hands gradually improved after withholding the use of lenvatinib mesylate (MK-7902) (since 20-DEC-2023). It was assessed to be related to hand foot syndrome. On 04-JAN-2024, the AE of hand foot syndrome was re-evaluated and updated to grade 2 which was tolerable. Starting from that day, lenvatinib mesylate (MK-7902) was resumed and the dose was reduced to 14 mg QD. From then on until C17D22, no new abnormalities were found in the physical examinations during each visit, and the trial participant continued to receive treatment with vitamin E urea ointment. On 09-JAN-2024, hemoglobin was 97 g/L. On 30-JAN-2024, hemoglobin was 102 g/L. On 20-FEB-2024, hemoglobin was 103 g/L. On 12-MAR-2024, hemoglobin was 109 g/L. On 03-APR-2024, hemoglobin was 115 g/L. On 22-APR-2024, hemoglobin was 104 g/L. On 13-MAY-2024, hemoglobin was 112 g/L. On 03-JUN-2024, hemoglobin was 106 g/L. On 25-JUN-2024, hemoglobin was 105 g/L. On 16-JUL-2024, hemoglobin was 106 g/L. On 06-AUG-2024, hemoglobin was 107 g/L. On 27-AUG-2024, hemoglobin was 104 g/L. On 21-NOV-2024, Bilateral knee joint X-ray was performed, and the result was: Degenerative osteoarthritis of both knees (abnormal). On 26-NOV-2024, left knee magnetic resonance imaging (MRI) was performed and the result was: Left knee osteoarthritis with multiple free bodies in the joint cavity; Wear of inner and outer meniscus, horizontal tear of outer meniscus; Anterior and posterior cross cutting band degeneration; Left knee joint effusion; Synovitis, edema of the infrapatellar fat pad and subcutaneous soft tissue in front of the patella (abnormal). On 28-NOV-2024, acupuncture was performed, and the result was: Degenerative arthritis of both knees and degenerative arthritis of the left knee (abnormal) and the result for magnet therapy was: Degenerative arthritis of both knees and degenerative arthritis of the left knee (abnormal). On 21-JAN-2025, hemoglobin was 98 g/L. On 10-FEB-2025, hemoglobin was 103 g/L. On 03-MAR-2025, hemoglobin was 93 g/L. On 24-MAR-2025, hemoglobin was 93 g/L. On 15-APR-2025, hemoglobin was 99 g/L. On 06-MAY-2025, hemoglobin was 103 g/L. On 27-MAY-2025, hemoglobin was 95 g/L. On 17-JUN-2025, hemoglobin was 95 g/L. On 08-JUL-2025, hemoglobin was 91 g/L. On 29-JUL-2025, hemoglobin was 96 g/L. On 19-AUG-2025, hemoglobin was 90 g/L. On the same day, physical examination at visit C18D1 revealed an increase in rash on the limbs (accompanied by erythema) compared to the previous visit, with new rash and erythema on the back, and scattered old rash and erythema on the chest. The rash area has peeling and flaking. Both upper limbs and back erythema were connected in patches. The trial participant complained of self-scratching due to itching, and after scratching, redness appeared around the rash. Based on the above situation, the original AE Hand Foot Syndrome has been revised to AE Atopic Dermatitis. Considering that the trial participant's AE atopic dermatitis was moderate to severe, the AE level has been updated to grade 3 since 19-AUG-2025. On the same day (19-AUG-2025), the trial participant was treated with dupilumab injection (DABITOL) 200mg/vial 400mg st. subcutaneously (SC) injected. The trial participant was advised to seek timely medical attention at a dermatology department (also reported as on a dermatologist). The use of lenvatinib mesylate (MK-7902) capsules has been suspended since 20-AUG-2025. On 21-AUG-2025, the last dose of pembrolizumab (MK-3475) was administered to the trial participant (Cycle 18). On 02-SEP-2025, physical examination revealed the addition of blisters on the hands and feet (compared to 19-AUG-2025), but no significant changes were observed in the rest. The trial participant was treated again with dupilumab (DABITOL) injection at a dose of 300mg SC. Then, the trial participant came to dermatology department for treatment. Physical examination showed extensive edematous erythema on the trunk and limbs, with some showing target type, swelling on the hands and feet, and tension blisters. The diagnosis was bullous pemphigoid, and the trial participant was planned to be admitted to the hospital for treatment. On 03-SEP-2025, pathological examination showed positive, Anti-BP180 antibody showed positive, Anti-BP230 antibody showed positive. The prescription drug was prednisolone acetate tablets (5mg/tablet), 40mg qd, orally and halometasone monohydrate (+) triclosan cream (XINSHIQUDE CREAM) apply an appropriate amount for external use, as a treatment. On 05-SEP-2025, the trial participant was admitted to the hospital due to the event. Since that day, prednisone acetate, borneol (+) chlorhexidine acetate (+) menthol (FU FANG CU SUAN LV JI DING PEN JI), ozone, minocycline hydrochloride, loratadine was given as treatment for event. On 07-SEP-2025, the last dose of belzutifan (MK-6482) was administered to the trial participant (Cycle 18). International normalized ratio (INR) was 1.65 (units not applicable) and 1.47 (units not applicable), prothrombin time was 17.9 seconds, Brain natriuretic peptide was 874 pg/mL, prothrombin time was 16.8 seconds, ultrasound of deep veins in both lower limbs showed No thrombus was found in the deep veins of both lower limbs (normal), Bilateral upper limb venous ultrasound showed No thrombosis in the veins of both upper limbs (normal), C-reactive protein was 13.95 mg/L, D-dimer was 12.76 mg/L and 14.11 mg/L, High-sensitivity C-reactive protein (hs-CRP) was 19.12 mg/L, Pulmonary ultrasound examination showed bilateral atelectasis, bilateral pleural effusion (abnormal), brain natriuretic peptide (BNP) was 862 pg/mL, aspartate aminotransferase was 1985 IU/L, calcium was 2.18 mmol/L, sodium was 141.3 mmol/L, potassium was 5.17 mmol/L,

hemoglobin was 59 g/L, chlorine was 99.8 mmol/L, magnesium was 1.03 mmol/L, alanine aminotransferase was 1283 IU/L, phosphorus was 1.46 mmol/L, and alkaline phosphatase was 157 IU/L. On the same day, finger oxygen saturation was measured at 59%-76% and nasal cannula oxygen was given at 2L/min. After retesting, finger oxygen saturation was 85%, and nasal cannula oxygen was given at 4L/min. Arterial blood gas analysis showed a P_{O2} of 56mmHg. It was recommended to transfer the trial participant to the intensive care unit and receive mask oxygen at 6L/min. On the same day, the trial participant was transferred to the intensive care unit. On the morning of 08-SEP-2025, at 08:16, it was learned that the trial participant was admitted to the dermatology ward of the hospital for treatment of bullous pemphigus vulgaris. Since that day, fluticasone propionate was given as treatment. Also, Prothrombin time (PT) was 15 seconds, INR was 1.39 (units not applicable), hs-CRP was 23.6mg/L, D-dimer was 12.38 mg/L, CT pulmonary angiogram was Multiple central distribution of ground glass density lesions, consolidation lesions, and cord lesions in both lungs, with large amounts of pleural effusion on both sides and adjacent lower lobe hypoplasia in both lungs. The heart is full, suggesting the possibility of heart dysfunction and pulmonary edema, which is more recent than before (anormal), physical therapy that promotes blood circulation in the lower limbs showed preventing venous thrombosis in both lower limbs (normal), Thoracic tube drainage showed Treatment of bilateral massive pleural effusion related to heart failure (abnormal), Brain natriuretic peptide was 681 pg/ml. On 09-SEP-2025, INR was 1.18 (units not applicable), D-dimer was 6.11 mg/L, hs-CRP was 38.86 mg/L, Brain natriuretic peptide was 412 pg/ml, PT was 12.7 seconds. On 10-SEP-2025, hs-CRP was 69.09 mg/L, Brain natriuretic peptide was 334 pg/ml, APTT was 25.8 seconds. D-dimer was 2.5 mg/L. On 11-SEP-2025, D-dimer was 1.18 mg/L, hs-CRP was 67.81 mg/L, Chest ultrasound showed bilateral pleural effusion (significant on the right side) (Abnormal), APTT was 26.1 seconds, Thoracic tube drainage showed Treatment of bilateral massive pleural effusion related to heart failure (abnormal), Brain natriuretic peptide was 170 pg/ml. On 12-SEP-2025, APTT was 24.9 seconds, hs-CRP was 40.06 mg/L, Brain natriuretic peptide was 164 pg/ml, D-dimer was 1.02 mg/L. On 14-SEP-2025, Brain natriuretic peptide was 148 pg/ml, and APTT was 24.5 sec. On 15-SEP-2025 at 13:35 (also reported as on the afternoon), the trial participant's family member (son-in-law) informed that after treatment, his condition has recovered steadily and has now improved. Physical examination that day (15-SEP-2025) showed no new skin lesions were observed on the limbs or trunk, with only extensive old brown pigmentation spots remaining. He reported no itching symptoms. Subsequently, he was discharged on the afternoon in the same date with a discharge diagnosis of bullous pemphigoid. The trial participant reported that the itching symptoms had completely disappeared on the day of discharge (on 15-SEP-2025). The specific diagnosis and treatment processed and medication dosage adjustment of the trial participant after admission were unknown, and further relevant information needed to be collected. Also, the AE Atopic Dermatitis has been revised to AE bullous pemphigoid. On that day, the evaluation of serious adverse event (SAE) bullous pemphigoid was updated as an unexpected serious adverse event, a follow up report on SAE bullous pemphigoid was submitted. The SAE follow-up and summary report would be submitted after further collection, summary, and organization of trial participant hospitalization data. Also on 15-SEP-2025, Ultrasound of deep veins in both lower limbs showed No deep veins were found in both lower limbs (normal), Bilateral iliac artery ultrasound showed no significant abnormalities (normal), and Bilateral lower limb arterial ultrasound showed Ultrasound of both lower limb arteries showed no significant abnormalities (normal). On 26-SEP-2025, Anti-BP180 antibody showed positive, Anti-BP230 antibody showed negative. it was learned that the trial participant came to our dermatology department for a follow-up visit (on 26-SEP-2025) (During communication with the trial participant on 30-SEP-2025 [AM 8:07]). The medical record showed that no skin lesions were found on the body, but brown deposition spots were left on the body. That day's physical examination showed no new skin lesions on the limbs and trunk, all of which were old brown deposition spots. On 29-SEP-2025, hemoglobin was 109 g/L and APTT was 23.8 seconds. He was still taking medication to treat AE bullous pemphigoid, and the AE grade was not adjusted yet, still at grade 2 (CTCAE 5.0). On 30-SEP-2025, the overall condition of the trial participant was comprehensively evaluated, and lenvatinib mesylate (MK-7902) was restored. Due to AE anemia (not reported as adverse event, conflicting information), the dose was reduced to 10 mg QD. Belzutifan (MK-6482) permanently discontinued due to AE type I respiratory failure (not reported as adverse event, conflicting information). On 11-OCT-2025, Anti-BP180 antibody showed positive and also negative (conflicting). On 21-OCT-2025, APTT was 23.5 seconds. The current medical history indicated "oral medication as per protocol". After communication and verification with the trial participant, "oral medication as per protocol" referred to the prednisolone acetate tablet regimen determined during the follow-up consultation at our dermatology department on 24-OCT-2025. Physical examination on 28-OCT-2025, showed no new skin lesions were observed on the limbs or trunk, with only extensive old brown pigmentation spots remaining. He reported no current itching symptoms. On 07-NOV-2025, the trial participant returned to our dermatology department for a follow-up consultation and prescription, which was learned upon communication with the trial participant 8:31 on 13-NOV-2025. The trial participant came to the hospital for a C19D22 follow-up visit on 13-NOV-2025. On 13-NOV-2025, physical examination revealed no new skin lesions on the limbs and trunk, with only large areas of old brown pigmentation remaining. The trial participant reported no current itching symptoms. Regarding the AE (Adverse Event) of bullous pemphigoid, there was no change between the physical examinations (on 13-NOV-2025, and 28-OCT-2025). The trial participant is currently undergoing oral and topical medication for bullous pemphigoid. According to the follow-up plan developed by the dermatologist for the trial participant, the next follow-up test for bullous pemphigoid-related antibodies is scheduled for the end of December 2025. Based on the existing medical records and the above information, it is assessed that the AE of bullous pemphigoid has not yet resolved and is still in the recovery phase. The AE level for this visit will not be adjusted and remains at Grade 2 (CTCAE 5.0). Since 01-DEC-2025, dose of prednisolone acetate was adjusted as 25 mg. On 03-DEC-2025, the last reported dose of lenvatinib mesylate (MK-7902) 10 mg was administered to the trial participant (Cycle 19).

At the reporting time, the trial participant recovered from the event of bullous pemphigoid (Grade 3).

The action taken with pembrolizumab (MK-3475) was reported as dose not changed, with belzutifan (MK-6482) was reported as drug interrupted with positive dechallenge results, while with lenvatinib mesylate (MK-7902) was reported as dose reduced with positive dechallenge result and negative rechallenge results.

The investigator considered the event of bullous pemphigoid (Grade 4) to be related to pembrolizumab (MK-3475), but to be not related to belzutifan (MK-6482) or lenvatinib mesylate (MK-7902), nor to the study procedure. It was also reported that according to the research manual, for safety reporting purposes, the expected serious adverse reactions of pembrolizumab (MK-3475) monotherapy include but are not limited to "bullous dermatitis, pemphigoid, and pemphigus". Therefore, SAE bullous pemphigoid was evaluated as an expected serious adverse event. The correlation with the experimental drug has been revised from not related to pembrolizumab (MK-3475), not related to belzutifan (MK-6482),

and related to lenvatinib mesylate (MK-7902) to related to pembrolizumab (MK-3475), possibly related to belzutifan (MK-6482) (final evaluation to be not related to belzutifan [MK-6482] in filed), and not related to lenvatinib mesylate (MK-7902).

The investigator considered the event of bullous pemphigoid (Grade 4) to be life-threatening (LT) event.

Follow-up information has been received from the investigator on 11-DEC-2025. The sign and symptoms regarding the causality of belzutifan (MK-6482) was updated.

APTT [APTT (10003064)*], 2025-09-10, 25.8 seconds (26.9 - 37.6) seconds

APTT [APTT (10003064)*], 2025-09-11, 26.1 seconds (26.9 - 37.6) seconds

APTT [APTT (10003064)*], 2025-09-12, 24.9 seconds (26.9 - 37.6) seconds

APTT [APTT (10003064)*], 2025-09-14, 24.5 seconds (26.9 - 37.6) seconds

APTT [APTT (10003064)*], 2025-09-29, 23.8 seconds (26.9 - 37.6) seconds

APTT [APTT (10003064)*], 2025-10-21, 23.5 seconds (26.9 - 37.6) seconds

Alanine aminotransferase [Alanine aminotransferase (10001546)*], 2025-09-07, 1283 IU/L (9 - 50) IU/L

alkaline phosphatase [Alkaline phosphatase (10001674)*], 2025-09-07, 157 IU/L (45 - 125) IU/L

Aspartate aminotransferase [Aspartate aminotransferase (10003476)*], 2025-09-07, 1985 IU/L (15 - 40) IU/L

Anti-BP180 antibody [Autoantibody test (10060972)*], 2025-09-03, Positive

Anti-BP180 antibody [Autoantibody test (10060972)*], 2025-09-26, Positive

Anti-BP180 antibody [Autoantibody test (10060972)*], 2025-10-11, Positive

Anti-BP180 antibody [Autoantibody test (10060972)*], 2025-10-11, Negative

Anti-BP230 antibody [Autoantibody test (10060972)*], 2025-09-03, Positive

Anti-BP230 antibody [Autoantibody test (10060972)*], 2025-09-26, Negative

BNP [BNP (10068024)*], 2025-09-07, 862 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-07, 874 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-08, 681 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-09, 412 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-10, 334 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-11, 170 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-12, 164 pg/mL (UNKNOWN - 100) pg/mL

Brain natriuretic peptide [Brain natriuretic peptide (10053406)*], 2025-09-14, 148 pg/mL (UNKNOWN - 100) pg/mL

C-reactive protein [C-reactive protein (10006824)*], 2025-09-07, 13.95 mg/L (UNKNOWN - 8) mg/L

CT pulmonary angiogram [CT pulmonary angiogram (10078646)*], 2025-09-08, Multiple central distribution of ground glass density lesions,consolidation lesions, and cord lesions in both lungs,with large amounts of pleural effusion on both sides and adjacent lower lobe hypoplasia in both lungs.The heart is full, suggesting the possibility of heart dysfunction and pulmonary edema,which is more recent than before. / ABNORMAL

Calcium [Calcium (10006948)*], 2025-09-07, 2.18 mmol/L (2.11 - 2.52) mmol/L

chlorine [Chloride (10008572)*], 2025-09-07, 99.8 mmol/L (99 - 110) mmol/L

chlorine [Chloride (10008572)*], 2025-09-09, 95.1 mmol/L (99 - 110) mmol/L

Dermatologic examination [Dermatologic examination (10058379)*], 2025-09-02, extensive edematous erythema on the trunk and limbs ,with some showing target type, swelling on the hands and feet ,and tension blisters. The diagnosis was bullous pemphigoid

Elevated eosinophils [Eosinophils (10014963)*], 2025-09-14, 1.35 10⁹/L (0.02 - 0.52) 10⁹/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-07, 12.76 mg/L (UNKNOWN - 0.24) mg/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-07, 14.11 mg/L (UNKNOWN - 0.24) mg/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-08, 12.38 mg/L (UNKNOWN - 0.24) mg/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-09, 6.11 mg/L (UNKNOWN - 0.24) mg/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-10, 2.5 mg/L (UNKNOWN - 0.24) mg/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-11, 1.18 mg/L (UNKNOWN - 0.24) mg/L

D-dimer [Fibrin D dimer (10016577)*], 2025-09-12, 1.02 mg/L (UNKNOWN - 0.24) mg/L

Hemoglobin [Hemoglobin (10019481)*], 2023-11-06, 103 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2023-11-27, 100 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2023-12-18, 100 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-01-09, 97 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-01-30, 102 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-02-20, 103 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-03-12, 109 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-04-03, 115 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-04-22, 104 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-05-13, 112 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-06-03, 106 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-06-25, 105 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-07-16, 106 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-08-06, 107 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2024-08-27, 104 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-01-21, 98 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-02-10, 103 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-03-03, 93 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-03-24, 93 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-04-15, 99 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-05-06, 103 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-05-27, 95 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-06-17, 95 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-07-08, 91 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-07-29, 96 g/L (130 - 175) g/L

Hemoglobin [Hemoglobin (10019481)*], 2025-08-19, 90 g/L (130 - 175) g/L

hemoglobin [Hemoglobin (10019481)*], 2025-09-07, 59 g/L (130 - 175) g/L

hemoglobin [Hemoglobin (10019481)*], 2025-09-29, 109 g/L (130 - 175) g/L

hs-CRP [High-sensitivity C-reactive protein (10082964)*], 2025-09-07, 19.12 mg/L (UNKNOWN - 3) mg/L

hs-CRP [High-sensitivity C-reactive protein (10082964)*], 2025-09-08, 23.6 mg/L (UNKNOWN - 3) mg/L

hs-CRP [High-sensitivity C-reactive protein (10082964)*], 2025-09-09, 38.86 mg/L (UNKNOWN - 3) mg/L

hs-CRP [High-sensitivity C-reactive protein (10082964)*], 2025-09-10, 69.09 mg/L (UNKNOWN - 3) mg/L

hs-CRP [High-sensitivity C-reactive protein (10082964)*], 2025-09-11, 67.81 mg/L (UNKNOWN - 3) mg/L

hs-CRP [High-sensitivity C-reactive protein (10082964)*], 2025-09-12, 40.06 mg/L (UNKNOWN - 3) mg/L

INR [INR (10022400)*], 2025-09-07, 1.65 (units not applicable) (0.88 - 1.1)

INR [INR (10022400)*], 2025-09-07, 1.47 (units not applicable) (0.88 - 1.1)

INR [INR (10022400)*], 2025-09-08, 1.39 (units not applicable) (0.88 - 1.1)

INR [INR (10022400)*], 2025-09-09, 1.18 (not applicable) (0.88 - 1.1)

Acupuncture [Investigation (10062026)*], 2024-11-28, Degenerative arthritis of both knees and degenerative arthritis of the left knee (abnormal)

Thoracic tube drainage [Investigation (10062026)*], 2025-09-08, Treatment of bilateral massive pleural effusion related to heart failure / ABNORMAL

magnet therapy [Investigation (10062026)*], 2024-11-28, Degenerative arthritis of both knees and degenerative arthritis of the left knee (abnormal)

physical therapy that promotes blood circulation in the lower limbs [Investigation (10062026)*], 2025-09-08, Preventing venous thrombosis in both lower limbs / NORMAL

Pulmonary ultrasound examination [Lung ultrasound (10078647)*], 2025-09-07, Bilateral atelectasis, bilateral pleural effusion (abnormal).

left knee MRI [MRI (10028049)*], 2024-11-26, Left knee osteoarthritis with multiple free bodies in the joint cavity; Wear of inner and outer meniscus, horizontal tear of outer meniscus; Anterior and posterior cross cutting band degeneration; Left knee joint effusion; Synovitis, edema of the infrapatellar fat pad and subcutaneous soft tissue in front of the patella (abnormal)

magnesium [Magnesium (10025430)*], 2025-09-07, 1.03 mmol/L (0.75 - 1.02) mmol/L

Oxygen saturation [Oxygen saturation (10033316)*], 2025-09-07, measured at 59%-76%

Oxygen saturation [Oxygen saturation (10033316)*], 2025-09-07, 85 %

P02 [P02 (10035766)*], 2025-09-07, 56 mm Hg

Pathological examination [Pathology test (10068056)*], 2025-09-03, Positive

phosphorus [Phosphorus (10034939)*], 2025-09-07, 1.46 mmol/L (0.85 - 1.51) mmol/L

Physical examination [Physical examination (10034986)*], 2023-10-11, scattered skin fissures on both hands and feet, accompanied by peeling

Physical examination [Physical examination (10034986)*], 2025-08-19, an increase in rash on the limbs (accompanied by erythema) compared to the previous visit, with new rash and erythema on the back, and scattered old rash and erythema on the chest. The rash area has peeling and flaking. Both upper limbs and back erythema are connected in patches. The patient complained of self scratching due to itching, and after scratching, redness appeared around the rash.

Physical examination [Physical examination (10034986)*], 2025-09-02, the addition of blisters on the hands and feet compared to 13-AUG-2025, but no significant changes were observed in the rest

Physical examination [Physical examination (10034986)*], 2025-09-15, no new skin lesions were observed on the limbs or trunk, with only extensive old brown pigmentation spots remaining

Physical examination [Physical examination (10034986)*], 2025-09-26, The medical record showed that no skin lesions were found on the body, but brown deposition spots were left on the body. Today's physical examination showed no new skin lesions on the limbs and trunk, all of which were old brown deposition spots.

Physical examination [Physical examination (10034986)*], 2025-10-28, No new skin lesions were observed on the limbs or trunk, with only extensive old brown pigmentation spots remaining

physical examination [Physical examination (10034986)*], 2025-11-13, no new skin lesions on the limbs and trunk, with only large areas of old brown pigmentation remaining. Regarding the AE (Adverse Event) of bullous pemphigoid, there was no change between the physical examinations on 13-NOV-2025, and 28-OCT-2025.

physical examinations [Physical examination (10034986)*], no new abnormalities were found in the physical examinations (From 04-JAN-2025 until C17D22)

Potassium [Potassium (10036439)*], 2025-09-07, 5.17 mmol/L (3.5 - 5.5) mmol/L

PT [Prothrombin time (10037056)*], 2025-09-07, 17.9 seconds (10.1 - 12.6) seconds

PT [Prothrombin time (10037056)*], 2025-09-07, 16.8 seconds (10.1 - 12.6) seconds

PT [Prothrombin time (10037056)*], 2025-09-08, 15 seconds (10.1 - 12.6) seconds

PT [Prothrombin time (10037056)*], 2025-09-09, 12.7 seconds (10.1 - 12.6) seconds

Sodium [Sodium (10041263)*], 2025-09-07, 141.3 mmol/L (137 - 147) mmol/L

sodium [Sodium (10041263)*], 2025-09-09, 135.41 mmol/L (137 - 147) mmol/L

Thoracic tube drainage [Thoracic cavity drainage (10053975)*], 2025-09-11, Treatment of bilateral massive pleural effusion related to heart failure / ABNORMAL

Chest ultrasound [Ultrasound chest (10052962)*], 2025-09-11, Bilateral pleural effusion (significant on the right side) / ABNORMAL

Bilateral iliac artery ultrasound [Ultrasound scan (10045434)*], 2025-09-15, Bilateral iliac artery ultrasound showed no significant abnormalities / NORMAL

Bilateral lower limb arterial ultrasound [Ultrasound scan (10045434)*], 2025-09-15, Ultrasound of both lower limb arteries showed no significant abnormalities / NORMAL

Bilateral upper limb venous ultrasound [Vascular ultrasound (10052965)*], 2025-09-07, No thrombosis in the veins of both upper limbs / NORMAL

Ultrasound of deep veins in both lower limbs [Vascular ultrasound (10052965)*], 2025-09-07, No thrombus was found in the deep veins of both lower limbs / NORMAL

Ultrasound of deep veins in both lower limbs [Vascular ultrasound (10052965)*], 2025-09-15, No deep veins were found in both lower limbs / NORMAL

Bilateral knee joint

X-ray [X-ray of thigh, knee & lower leg (10048177)*], 2024-11-21, Degenerative osteoarthritis of both knees (abnormal)

Based on the clinically relevant information currently available for this individual case, the known safety profile of pembrolizumab, the nature of the event, and the time of occurrence, the reported event of Pemphigoid is considered related to pembrolizumab but not related to belzutifan or Lenvatinib. The biological plausibility, time to onset, no other etiology, and improvement with corticoid treatment support the possible causal association.

No changes to the pembrolizumab, belzutifan, and lenvatinib mesylate safety information are warranted at this time. Merck Sharp & Dohme LLC, continues to monitor the safety profile of pembrolizumab and belzutifan. Eisai continues to monitor the safety profile of lenvatinib mesylate.

II. Suspect Product(s) Information

#1) 14. Suspect Product(s): BELZUTIFAN Tablet (belzutifan) - Batch/Lot #: ;

- 15. Daily Dose(s): 120 mg 120 mg QD, every 1 days; 120 mg 120 mg QD, every 1 days
- 16. Route(s) of Administration: Oral; Oral
- 17. Indication(s) For Use: Advanced Clear Cell Renal Cell Carcinoma (ccRCC) [Clear cell renal cell carcinoma (10073251)*]
- 18. Therapy Date(s) (from/to): 18-Oct-2023 to 07-Sep-2025; 07-Sep-2023 to 17-Oct-2023
- 19. Therapy Duration: 1.89 years; 41 days

#2) 14. Suspect Product(s): LENVATINIB MESYLATE Capsule (lenvatinib mesylate) - Batch/Lot #: ; ; ;

- 15. Daily Dose(s): 14 mg 14 mg, qd (dose reduced), every 1 days; 10 mg 10 mg, qd (dose reduced), every 1 days; 20 mg 20 mg, qd, every 1 days; 20 mg 20 mg, qd, every 1 days
- 16. Route(s) of Administration: Oral; Oral; Oral; Oral
- 17. Indication(s) For Use: Advanced Clear Cell Renal Cell Carcinoma (ccRCC) [Clear cell renal cell carcinoma (10073251)*]
- 18. Therapy Date(s) (from/to): 04-Jan-2024 to 19-Aug-2025; 30-Sep-2025 to --; 07-Sep-2023 to 17-Oct-2023; 18-Oct-2023 to 19-Dec-2023
- 19. Therapy Duration: 1.627 years; ongoing; 41 days; 63 days

#3) 14. Suspect Product(s): PEMBROLIZUMAB Solution for injection (pembrolizumab) - Batch/Lot #: ;

- 15. Daily Dose(s): 400 mg 400 mg, Q6W up to 18 cycles, every 6 weeks; 400 mg 400 mg, Q6W up to 18 cycles, every 6 weeks
- 16. Route(s) of Administration: Intravenous (not otherwise specified); Intravenous (not otherwise specified)
- 17. Indication(s) For Use: Advanced Clear Cell Renal Cell Carcinoma (ccRCC) [Clear cell renal cell carcinoma (10073251)*]
- 18. Therapy Date(s) (from/to): 07-Sep-2023 to 07-Sep-2023; 18-Oct-2023 to 21-Aug-2025
- 19. Therapy Duration: 31 minutes; 1.844 years

III. Concomitant Product(s) And History

23. Other Relevant History (e.g. diagnostics, allergies, pregnancy with last month of period, etc.):

Advanced Clear Cell Renal Cell Carcinoma (ccRCC) [Clear cell renal cell carcinoma (10073251)*], (continuing)

Assessment Results

- PEMBROLIZUMAB Solution for injection ↔ Bullous pemphigoid, Investigator, Global Introspection, Related
- PEMBROLIZUMAB Solution for injection ↔ Bullous pemphigoid, Sponsor, Global Introspection, Related
- LENVATINIB MESYLATE Capsule ↔ Bullous pemphigoid, Sponsor, Global Introspection, Not Related
- LENVATINIB MESYLATE Capsule ↔ Bullous pemphigoid, Investigator, Global Introspection, Not Related
- BELZUTIFAN Tablet ↔ Bullous pemphigoid, Investigator, Global Introspection, Not Related
- BELZUTIFAN Tablet ↔ Bullous pemphigoid, Sponsor, Global Introspection, Not Related